Registered number: 2073465
Charity number: 519332

Mental Health Concern

(A company limited by guarantee)

Trustees' report and financial statements

31 March 2016

Mental Health Concern

(A company limited by guarantee)

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Mental Health Concern

(A company limited by guarantee)

Reference and administrative details
Year ended 31 March 2016

Trustees	Mrs E A Robinson, Chair Reverend C Brown, Vice Chair Mr D Arthur Ms R Granger Mrs Z Jones Mr R B Sharpe (resigned 16 December 2015) Mr D Smith Dr A Walsh Mr R McEvoy (appointed 16 December 2015)	
Company registered number	2073465	
Charity registered number	519332	
Registered office	Buttress House 36 Brenkley Way Seaton Burn Newcastle upon Tyne NE13 6DS	
Executive team	Chief Executive	Brendan Hill
	Director of Services - Mental Health Concern	Scott Vigurs
	Director of Services - Insight Healthcare	Richard Carlton-Crabtree
	Director of Corporate Services and Finance	Gordon Burns
	Director of Human Resources	Claire Farnell
	Director of Business Development	Liam Gilfellon
Independent auditors	UNW LLP Chartered Accountants Citygate St James Boulevard Newcastle upon Tyne NE1 4JE	
Bankers	Lloyds Bank plc High Street Gosforth Tyne and Wear NE3 1JQ	

Advisers (continued)

Solicitors	PGS Law Law Court Chamber Waterloo Square South Shields Tyne and Wear NE33 1AW Muckle LLP Times Central 31 Gallowgate Newcastle upon Tyne NE1 4BF

The trustees (who are also directors of the charity for the purposes of the Companies Act) present their annual report together with the audited financial statements of Mental Health Concern (the company and the group) for the year ended 31 March 2016. This is our 30th year of operation and we will be celebrating this landmark later this year.

The trustees confirm that the annual report and financial statements of the company and the group comply with the current statutory requirements, the requirements of the company and the group's governing document and the provisions of the Statement of Recommended Practice (SORP), applicable to charities preparing their accounts in accordance with the Financial Reporting Standard applicable in the UK and Republic of Ireland (FRS 102) (effective 1 January 2015).

Strategic Report

Objectives and Activities

Policies and objectives

Mental Health Concern (known as MHC) sits alongside its subsidiaries as part of "Concern Group".

Our mission is succinct and to the point: "to improve the mental health and wellbeing of the people we serve".

We are driven by our strong value base, which underpins the way we work with our service users, staff, service commissioners and partners. We value:

- compassion and hopefulness,
- being open and friendly,
- inclusivity and fairness,
- experience and expertise,
- hard work, creativity, innovation,
- going the extra mile with people to achieve the right outcomes

Strategies for achieving objectives

Our continued success over 30 years of operation is testament to our ability to develop and adapt to the evolving needs of the people we serve, and of those who commission us. We continually strive to consolidate our position as a key not-for-profit provider of NHS services, annually supporting thousands of people with a wide range of mental health conditions.

The group specialises in:

- 24 hour nursing services for those with dementia including respite and assessment, and a community challenging behaviour team.
- 24 hour nursing services for those with complex mental health problems, supporting their rehabilitation and recovery.
- Supported housing services.
- Community and well-being services which support meaningful activity and employability.
- Through our subsidiary, Insight Healthcare Limited (IH), we provide employee wellbeing services and primary care mental health services, centred on the delivery of a range of psychological therapies.

- Hosting and support for smaller mental health-related projects.

Our management structure is designed to support our frontline services, to provide corporate effectiveness and to meet new and ongoing commissioning or service development opportunities. The executive team consists of the Chief Executive and five directors, ensuring our clinical and service-driven ethos is maintained and supported by appropriately strong support services. These include financial and legal governance, training and workforce development.

Activities for achieving objectives

The trustees have continued to review and develop the way we work and how best to lead the group in a continuingly difficult commissioning environment.

- The Articles of Association have been amended to increase the maximum permitted number of trustees, allowing us to respond to any need to strengthen an area of expertise.
- A new role of associate trustee has been developed, allowing us to recruit independent members of our committees and providing opportunities for potential trustees to work with us prior to committing to trusteeship.
- Our three-year business plan was approved and updates have been presented to the board throughout the year.
- A main strand of our plan is to continue to create efficiencies and better governance through the development of group-wide policies, procedures and processes. This year has seen the successful consolidation of two formerly separate finance teams, operating on separate systems, to create a single multi-functional department.
- A group environmental strategy has been developed and approved and the development of a group business continuity policy is underway.

We continue in our endeavours to improve our service provision, policy, and practice, in order to keep pace with the ever-changing demands of health and social care. We work alongside other providers, and with statutory commissioners, to develop and maintain "joined up" person centred services. Our position as a key provider of NHS-funded services continues and has further developed across England.

Our services continued to comply with, and often exceed, national monitoring standards and performance monitoring expectations set by our commissioners.

To meet the changing needs of our residential service users, we have been exploring how our properties can be best utilised in the future. This year, we have invested in professional surveys to provide detailed site plans and updated property valuations. To comply with statutory requirements, we also carried out an Energy Savings Opportunity Scheme (ESOS) survey in January 2016.

The results of the property valuations have been incorporated into our financial statements. The site plans and the ESOS survey results are helping to inform our considerations for the future development of our properties. The recommendations arising from the ESOS survey, to replace older equipment with more efficient newer items when appropriate, have been built into the actions of our environmental policy for the coming years.

An annual staff survey is conducted allowing us to measure overall staff satisfaction responses are valuable in illustrating employee wellbeing and helping us to identify areas of good practice and areas we need to target for improvement. We use the results to inform the workforce strategy, particularly in the areas of staff training, education, development, and wellbeing.

We have embraced the challenges posed by the increasing importance of information sharing, electronic records, interoperability, and digital inclusion, with a range of exciting initiatives that will take shape in the year ahead. Investment has been made to upgrade our systems and data security, replacing local server hardware with our networked systems. This work has allowed us to complete the national implementation of the electronic case management system for Insight Healthcare.

Building on the success in Insight Healthcare, we have completed work to capture all systems and information flows across the various service areas in Mental Health Concern. We found considerable duplication of effort and opportunities to simplify our processes, which we are developing during 2016/17. We have also commissioned the development of a unified IT database system, which we are aiming to deploy in the final quarter of 2016/17. As we develop this system, we will remain mindful of the interoperability agenda and ensure that we contribute fully to local, regional, and national initiatives.

We were pleased to have achieved Information Governance accreditation at level 2 in March 2015 for Mental Health Concern. Our submission for 2016/17 shows we have built on a number of areas and have increased our attainment level to 68%. Insight Healthcare has been accredited for a number of years and has maintained its own level 2 accreditation.

Main activites undertaken to further the charity's purposes for the public benefit

The trustees have considered the Charity Commission's guidance on public benefit. Our activities are described above and are undertaken to further our charitable purposes for the public benefit.

Achievements and performance

Key financial performance indicators

	2016	2015	2014
Change in Revenue %	12.2%	3.6%	17.6%
Change in Revenue £	£2.06m	£0.58m	£2.43m
Change in Charitable Expenditure %	11.3%	3.4%	20.9%
Change in Charitable Expenditure £	£1.89m	£0.55m	£2.79m
Workforce	577	462	446

Review of activities

Mental Health Concern

In our thirtieth year of operation, the range of our services is evidence of our ability to develop and adapt to the evolving needs of the people we serve and those who commission our services.

Older People's Services

We care for people who, because of their dementia, behave in ways that can be challenging or hard to understand. Our main focus is on getting to know the person as well as we can, so that we can meet their needs in ways that improve wellbeing. This is because we know that a person with dementia is just that – a person.

In Gateshead we provide four types of dementia services:

- 24-hour specialist nursing care
- respite
- short-term assessment (as an alternative to hospital admission)
- community challenging behaviour support

Rehabilitation and Recovery Services

Our specialist, 24-hour, nurse-led rehabilitation and recovery services help people with complex mental health problems to move from secure and hospital-based settings into the community. We also provide short-term relapse prevention packages as an alternative to hospital admission.

Our rehabilitation and recovery services focus on promoting:

- positive move-on within one to three years
- relapse prevention
- employment and increased employability
- access to education
- living skills
- condition management

Supported Housing

Our supported housing service helps people with complex mental health problems to move out of hospital or 24-hour care and take up a supported tenancy in the community. It is a highly-specialised service with tiers of support and supervision, which gives people who have often spent many years in institutional environments the opportunity for independent living and social inclusion.

Supported housing consists of four specific services:

- Newcastle Supported Housing
- Gateshead Supported Housing
- EIP (Early Intervention in Psychosis) Supported Housing
- Step Ahead Housing

We have invested this year in an additional property for the Step Ahead service and the financial valuations of our properties have been established to develop and fund our strategy for further supported housing provision.

Community and Wellbeing Services
Mental Health Concern's community services help over 1,000 people each year to get back into work, education, or increase meaningful activity. We help people to rebuild their social support networks and develop friendships. We stick by them to 'navigate' through the sometimes complex health and social care system, using our knowledge of what's out there to link people in.

Moving Forward
Our Moving Forward services in Newcastle, Gateshead, and South Tyneside can be accessed by anyone who has experienced mental health issues. The service focuses on linking people up with activities, groups and community resources, with the aim for them to:

- rediscover self confidence
- learn new skills or rediscover old interests
- build a stronger social network and have more people to do things with
- get back into education or training
- find paid or voluntary work

Fulfilling Lives
The Fulfilling Lives programme is a Lottery-funded partnership between Mental Health Concern, Changing Lives, and Aquilla Way. Together, we help adults who are likely to experience issues relating to three or more of the following:

- mental ill-health
- homelessness
- drug and alcohol issues
- offending

Mental Health Carer Support
In South Tyneside, we provide support to people who have caring responsibilities for people with mental health problems. We work in partnership with other professionals to provide emotional and practical help, as well as helping people to find the support they need in their caring roles.

Ways to Wellness
Ways to Wellness is a social prescribing service for people with long-term health conditions in the west of Newcastle. The service aims to improve its users' quality of life and reduce their use of mainstream health services, by enabling them to lead healthier lives and better manage their conditions. This is achieved by the provision of Link Workers, who help patients to identify and work to overcome the current barriers to managing their long-term health conditions.

Chain Reaction
Chain Reaction launched in 2015 as a new service commissioned by Newcastle City Council, designed to support older people (55+) to live independently in the community. By promoting the use of community-based resources and facilities, Chain Reaction encourages people to lead happier, healthier lives. It supports and sustains friendships in order to combat social isolation and to increase good emotional health and wellbeing.

Care Quality Commissioned Regulated Services Performance

Mental Health Concern operates seven CQC-regulated locations, within which we operate nine services. The reports from our most recent CQC inspections through 2015/16, under the new regulatory and inspection regime, have been very pleasing. The areas identified for improvement at Pinetree Lodge have been addressed through an action plan with CQC commenting in July 2016 "We found the provider had met the assurances they had given.

Service	Alderwood	Briarwood	Pinetree Lodge	Coalway Lane	Oakwell	Jubilee Mews	McGowan Court
CQC Status	Good	Good	Requires improvement	Good	Good	Good	Good
Date of last inspection	March 2016	December 2014	June 2015 (followed up June 2016)	January 2016	February 2016	March 2016	February 2016

Recovery focused key outcomes
During 2015/16 we supported 25% of our rehabilitation and recovery service users to achieve a significant employment/employability outcome, as well as supporting 22% of our service users to achieve an educational outcome.

We also supported 30% of people to move on into more independent accommodation this year.

Social outcomes	% service users supported to achieve outcomes	
	2016	**2015**
Employment/employability	25%	23%
Education/educational	22%	33%
Interests, creativity and expression	83%	100%
Moving on to more independent accommodation	30%	21%
Develop or improve independent living skills	100%	100%

Physical health outcomes
There is compelling evidence that people with a mental illness or a learning disability die younger than the general population.

It is well established that people with mental health problems are at an increased risk of adverse physical health outcomes. Among people with a severe mental illness (SMI), such as schizophrenia, an estimated 60% of excess mortality is due to physical illness. A person with an SMI is more likely to have a co-morbid physical health problem, and is more likely to die of that illness within five years, when compared to the general population.

This is why we continue to focus on the physical health of our beneficiaries. We will continue to work on enabling people to take more exercise this year and, alongside our 2014/15 'Good food on a Budget' initiative, hope to increase the number of people we support to maintain a healthy BMI.

Physical health outcomes	% service users supported to achieve outcome	
	2016	2015
Number of those who smoke who are provided with health education and smoking cessation information	100%	100%
Number of people supported to increase regular physical exercise	44%	46%
Number of people supported to make positive changes to their BMI, if required	45%	54%
Number of people supported to maintain an appropriate BMI	52%	40%
Percentage of residents with an annual physical health check	100%	96%
Number of those who have engaged in smoking cessation activities and have reduced/ceased smoking	23%	100%

Developing our approaches in wellbeing
There is evidence that mental wellbeing is a good indicator of how people are able to function and thrive. Past research and practice surrounding mental health and wellbeing have focused on mental health problems and on the prevention of developing a mental disorder (mental health problem), rather than on mental wellbeing. However, generally within mental health services we do not know much about mental wellbeing or its trends over time. We are proud of our continuing joint work with Northumberland, Tyne & Wear NHS Trust and Tees, Esk & Wear Valley NHS Trust in developing these approaches.

The Warwick-Edinburgh Mental Wellbeing Scale (WEMWBS) is a scale which has been validated for the measurement of mental wellbeing among people aged 13 to 74 in the UK (the general population). It comprises 14 positively-worded statements with five response categories, from 'none of the time' to 'all of the time'.

This year we published the first of our papers, evaluating the reliability and sensitivity of the WEMWBs measure for people in secondary mental health services. We have found that the tool is a reliable measure of wellbeing in this client group. With support from the North East Strategic Clinical Network, we have commissioned a project to develop a Positive Psychology and Wellbeing programme, jointly with people who have lived experience of poor mental health. We hope to evaluate the programme's effectiveness in 2016/17.

Support for the voluntary sector
MHC continues to support smaller voluntary sector projects and maintains the provision of groups such as Launchpad and North Tyneside's User Development project, both 'user-led' services which promote and enable those with mental health problems to have a voice in all aspects of mental health services.

Our Insight Healthcare services
Insight has continued to grow and, over the course of the year, we have mobilised and embedded new services whilst pursuing a strategy of greater integration within local healthcare systems in our longer standing services.

Referral volumes in 'Any Qualified Provider' localities have continued to rise steadily, as Insight has increasingly become the provider and partner of choice for GPs and other referrers in these areas, in part because of our consistently strong clinical outcomes and minimal waiting times.

Some 40,000 people accessed Insight's NHS commissioned primary care talking therapy services over the course of the year. Insight's ability to manage this growth, whilst maintaining our quality standards, has been underpinned by the completion of our programme to roll out 'Insight+'; our bespoke, web-based Case Management System. All Insight services (with the exception of some Wellbeing at Work and partnership services) are now using Insight+ which allows for greater visibility of all aspects of case management throughout the country and greater ease of use for staff, especially those working peripatetically in GP surgeries and other community venues away from Insight's offices.

The introduction of Insight+ and its web-based functionality also created the conditions needed to establish an Insight 'National Team', an initiative which was launched towards the end of 2015. The National team is based at our head office and has already begun supporting Insight's services throughout the country, enabled by the ability to access all localities' case records in real time via Insight+. It is envisaged that the National Team will continue to develop, offering increasing administrative and telephone-based clinical capacity to flexibly support all Insight services according to need. This is already helping to manage spikes in demand and the unpredictable nature of referral volumes in AQP areas.

Outcomes
Insight has continued to provide high-quality talking therapy services nationally. All IAPT services are required to provide full activity submissions to the Health and Social Care Information Centre (HSCIC) and this data is used to report a common set of KPIs that CCGs and provider organisations are measured against. This allows for nationally validated data, measuring individual service performance against a range of clinical and service level outcomes. Insight's 2015/16 performance against the principal KPIs is shown below.

Clinical outcomes have consistently exceeded both our contractual targets (nationally mandated at a 50% Moving to Recovery Rate) and the HSCIC reported all provider national average.

Clinical Outcomes	IAPT Services 2015/16
Proportion of service users 'Moving to Recovery'*	53%
Proportion of service users showing material improvement	71%

Shows clinical performance measured according to the national 'Moving to Recovery' definition.

Waiting time standards vary between services, but levels of access are strong across all localities with all service users treated within the NHS 18-week limit and the vast majority seen much more quickly, typically within one week of referral.

Waiting times	IAPT Services 2015/16
Accessing therapy within four weeks	71%
Assessed within 1 week	86%

Self-reported service user experience has been overwhelmingly positive.

Service User Satisfaction	IAPT Services 2015/16
Proportion of service users reporting high levels of satisfaction**	96.8%
Friends and Family test positive outcomes***	94.9%

*** Shows the proportion of service users completing the nationally specified DoH (IAPT) patient experience questionnaires and rating their experience of Insight's services within the highest two categories.*
**** Shows the proportion of service users answering 'likely' or 'very likely' in response to the NHS Friends and Family test: "How likely is it that you would recommend Insight Healthcare to a friend or family member?*

New services in Bassetlaw and Sefton
Insight successfully launched new NHS commissioned services in both Bassetlaw and Sefton at the start of 2015/16. Both mobilisations involved the TUPE transfer of a formerly NHS workforce and both have shown improved outcomes as the year progressed, as we have worked to inculcate Insight's processes and systems. The Sefton service is provided in partnership with the Cheshire and Wirral Partnership NHS Foundation Trust (CWP).

Integration and consolidation of Talking Therapy Services in Newcastle
We have gained a mandate from our commissioners in Newcastle to integrate 'Newcastle Talking Therapies', the IAPT service provided by Insight and NTW, with the primary care psychological therapy service provided by the Newcastle upon Tyne Hospitals NHS Foundation Trust (NuTH), to provide a single service with an innovative, joined-up model along 'Alliance Contracting' principles. The integrated service is called 'Talking Helps Newcastle' and offers an improved service user pathway with greater depth of capacity, for the benefit of service users and referrers.

Ongoing evolution of Insight's Wellbeing at Work offer
Clinical outcomes, waiting times, and service user experience have all continued to be very positive and ahead of target performance levels in Wellbeing at Work services. Insight won a major new commission in the year to provide substantive therapy to Members of Benenden Health, a not-for-profit healthcare mutual and long term partner of Insight. This service commenced in January 2016 and is expected to grow over the course of the coming year as an innovative provision helping to incorporate Wellbeing at Work benefits within more holistic psychological therapies and 'bridge the gap' with NHS commissioned services.

Quality Accreditation
Insight has maintained our BACP and ISO 9001:2008 accreditations after passing in year renewals; Insight continues to be recognised by commissioners, referrers and accrediting bodies as a safe and effective provider of primary care psychological therapies.

Financial review

Financial perfomance

Our increase in turnover reflects the two new IAPT services in Bassetlaw and Sefton, and the first year of our involvement in Ways to Wellness. Through competitive tendering we sadly lost our Northumberland IAPT service during 2015, and this is likely to reduce our level of income for the coming year.

All of our services, like those of many other charitable organisations, have continued to share the pressures borne by our various commissioners. We rely entirely on income from commissioned services and have not historically raised funds directly from the public, but we are beginning to look at these alternative sources of income to ensure we can meet the increasing needs of our service users.

The cost of our charitable activities has increased in line with our income. We provide care and support to people, and our biggest resource is our workforce. Our continued success is due to the hard work, dedication and skills of our staff, and the quality of their care is the reason we are well regarded by our commissioners.

Our balance sheet remains strong. We have changed the way we show the operational software products we have developed, showing them as intangible assets, in accordance with changes in reporting required by the new Charity SORP.

The existing use valuations of our freehold and long leasehold properties included the reduction in the value shown of our largest care home. The new value has been reflected in the results for the year and the implications of this were considered by the board. No need to further reduce the value of any of the assets through impairment is seen as necessary. The results of the ESOS survey showed our properties to be in good order and efficient for their age.

Our property and cash assets provide us with the platform for the future development of our services and for Concern Group to promote, improve, and advance health in general and mental health in particular.

Going concern

After making appropriate enquiries, the trustees have a reasonable expectation that the company has adequate resources to continue in operational existence for the foreseeable future. For this reason they continue to adopt the going concern basis in preparing the financial statements. Further details regarding the adoption of the going concern basis can be found in the Accounting Policies.

Financial risk management objectives and policies

Within the Strategic Risk Register, reviewed quarterly by the trustees, the most significant financial risk facing Concern Group is the reliance on commissioned public contracts in an economic period of austerity, and the impact of inflation risk. NHS commissioners are required to look for annual efficiencies in the costs of their services.

All members of Concern Group are subject to the risk of increases in purchase price of supplies. Where possible, prices are agreed in advance through the use of supplier quotations and tendering.

Our budgeting and forecasting processes are designed to alert management and trustees to potential problems.

Trustees review liquidity through regular reports including income and expenditure accounts, balance sheets and cash flow forecasts. To date, our liquid assets for the group have been sufficient to mitigate the impact of quarterly funding from the NHS and the impact of payments in arrears for relevant IAPT services. There are sufficient physical assets available to the trustees should the need arise to raise funds, through either borrowings or disposal, to remedy the situation without compromising operations.

We do not make charges to the individuals for the services we provide and as such have no exposure to credit risk in relation to service users and beneficiaries. Funding from the NHS and local authorities is based on commissioned services under standard public sector contracts. As this funding is derived from government related public bodies, it is considered that credit risk is minimal.

Each member of Concern Group has interest bearing assets, in the form of cash deposits, which are subject to changes in interest rates. The poor levels of interest have been recognised in our investment policy, but the need for sufficient operating funds to meet liquidity risk requires significant levels of cash to be held.

Principal risks and uncertainties

We have an extensive range of policies and procedures in place, coupled with the requisite training, clinical supervision, and support to mitigate potential risks. A strategic risk register is maintained, reviewed and updated by the Executive Team, the Governance Committee and then the Board. The controls put in place provide a reasonable, but not absolute, assurance that risks have been adequately mitigated

The principal risks highlighted by our review and monitoring are:

- We work with some very vulnerable individuals and we recognise the risks involved in delivering our services: to service users themselves, their families, our staff, and to the general public. We have clinical policies, procedures, supervision and training that are regularly updated and reviewed to ensure our practices and services minimise potential harm.
- Our services are commissioned and we recognise the financial pressures on public funded services as well as the constantly changing commissioning environment. The executive team work closely with all commissioning groups to ensure that these risks to the charity are minimised. All new services and re-tenders are subject to financial and operational review to ensure our standards of delivery are both viable and clinically sound.
- We operate alongside the NHS and other NHS-commissioned services and face competition to recruit and retain professionally trained, experienced and knowledgeable staff. Our staff understand the financial position this puts us in and have accepted our attempts to ensure that we have an efficient cost base. We also understand the pressures faced by our workforce and we try to ensure our remuneration packages are competitive.

Reserves policy

The trustees have considered a risk-based reserves policy, taking into account the issues raised by the strategic risk register, which has in turn fed the investment policy. The trustees have established

the need to increase the level of free reserves to £1.8m from £1.0m, to provide additional security against risks that may arise in future. The approved three-year business plan includes the aim of achieving an annual surplus from operations, through growth of services and increasing operating efficiencies. We recognise achieving our target will take a number of years, as we manage the pressures of increasing costs of resources and the available income from the tightening commissioning environment.

The trustees decided that it is prudent to maintain all its reserves as undesignated for the year ended 31 March 2016. Restricted reserves have been reviewed and, where services that were backed by restricted reserves are now considered part of the mainstream services of the charity, these reserves have been released to general funds.

The trustees are satisfied that both the policy on reserves and the current level of reserves are sufficient to cover risks to the charity at this time.

Investment policy and performance

The trustees have the power to invest and deal with the monies of the charity not immediately required for the purpose of its business, in such investments or securities and in such manner as may from time to time be determined.

The trustees have held investments, other than investments in property for the use of operational services, as cash on deposit.

An investment policy was considered during the year, which sets out the need to maintain sufficient working capital to support operations, but also highlighted the opportunity to invest cash into better-performing deposits or into assets to further the objectives of the charity. Investments into equities were considered inappropriate at this time, due to the level of funds available for investment and the potential volatility in value of such investments.

The trustees have supported the use of available funds to acquire further Step Ahead housing, to allow service users the opportunity to live in their own home, supported by our professional team. The rental return from this type of investment will be greater than leaving the funds on deposit. The first of these new properties was acquired during the year and the two service users have settled into their new home.

Plans for future periods

Future developments

Our position as a prominent figure in the development of mental health and wellbeing services has continued to develop and flourish throughout the year. We have maintained and developed our reputation as an organisation that actively engages in partnership working with our statutory and third sector colleagues for the benefit of the people we serve.

Our development as an organisation has enabled us to prosper in the continually-evolving mental health and wellbeing service arena, particularly in relation to the transformation in commissioning and regulatory bodies.

We have made significant steps in developing our profile as an innovative and progressive organisation which can act as a constructive partner in meeting the challenges of service provision in the years ahead, including ways to develop integrated services with health and social care provider partners. Providing better person-centred and connected services for the benefit of service users, communities, and our commissioners will be an ongoing priority. Our future strategic plans are cognisant of the demand for further integration around the person, which is also grounded in our charitable objectives and values.

We will hold on to our belief in promoting 'valued lives', which has been at the heart of what we do for many years and has played such a key role in our success. We will build upon these strong foundations, so that we can continue to shape our future with confidence.

Structure, governance and management

Constitution

Mental Health Concern ("MHC") is a company limited by guarantee number 2073465, incorporated on 12 November 1986 and is governed by its articles and memorandum. It is a registered charity with the number 519332.

Name	Company Number	Charity Number	Status
Mental Health Concern	2073465	519332	Limited by guarantee
Insight Healthcare Limited	4391008	n/a	Limited by shares and social enterprise
MHC / NTW LLP	OC360242	n/a	Limited liability partnership
MHC Resources Limited	6390967	n/a	Limited by shares and dormant

MHC promotes, improves, and advances health in general and mental health in particular, including the prevention and treatment of mental ill-health via a wide range of commissioned services. It provides treatments and interventions, social housing and support, without distinction as to an individual's age, race, gender, sexual orientation, political, religious, or other persuasion. The charity actively promotes service improvement and development in the field of mental health and emotional wellbeing. The charity's business mission statement upholds that 'as a responsive and enabling organisation, we will endeavour to provide distinctive and effective services.'

Method of appointment or election of trustees

The Articles of Association limit the number of trustees to nine individuals, increased from eight following a resolution approved on 26 August 2015.

The structure of the trustees' meetings allows them to discuss strategic issues and consider their performance during the year. It is at these meetings that the trustees consider whether they require additional trustees with experience and knowledge that is not currently available.

The Articles of Association set out the process for appointment of new members. A board member would notify MHC of the nomination(s) for election to the board in writing, not less than three days before each annual general meeting (AGM). At the AGM, the board would elect the member nominated and also elect a chair and a vice-chair.

Where it is deemed necessary to elect a new member to the board at a meeting other than the AGM, the nominated person would be proposed by a member and seconded by another member of the board of directors. This would follow discussions surrounding the nominee's credibility and suitability for a place on the board. The board of directors as a body is solely responsible for the election of any new members.

With the knowledge that Brian Sharpe intended to retire from the board, an advert was placed inviting those interested in joining to express their interest. Interviews took place and two new potential members were invited to attend board committee meetings to familiarise themselves with the organisational structure of MHC. One of the potential new trustees found it difficult to provide the commitment to regular meetings and decided not to join us. We are delighted though to welcome Richard McEvoy as a trustee and we are sure his knowledge and experience will help us to take Concern Group forward.

This year saw the retirement of Brian Sharpe from the board after nearly six years as a trustee. We thank him for his work and support over the years and wish him well in his new ventures.

All new trustees receive informal induction from member of the executive team.

Pay policy for senior staff

All employees of Concern Group, including all senior staff, are remunerated in accordance with an approved pay structure, with all roles now subject to job evaluation at creation or when changes in responsibilities are required of the post holder. Employees are appointed to a salary band, which includes a range of salaries to reflect the experience and skills highlighted in the job evaluation. The range of salaries allows employees to incrementally increase their earnings towards the top of their salary band subject to performance. Employees undertake regular performance reviews with their line managers. Proposed cost of living awards are presented to the trustees for approval before the start of each financial year.

Organisational structure and decision making

The governing body of the charity is the board of trustees which has met formally six times this year, with additional informal meetings when required and a full-day meeting to specifically discuss the business plan.

During the year, the board considered the need for additional members, including additional membership of the committees. The Articles of Association were amended to increase the maximum number of trustees from eight to nine. A new role of associate trustee was established to recruit additional non-executive members to the committees, allowing greater strategic input to the management of affairs.

There are four committees focusing on Clinical Governance/Service Delivery, Finance, Human Resources, and Governance. Information is provided by requisite managers to support committee

members and the board in their role to provide oversight, governance, and advice to the organisation. Every trustee is a member of at least one committee. The Governance Committee has a membership of the Vice Chair, in addition to the Chair of each of the three other committees. Executive decision making and day-to-day management is vested in a professional team led by the Chief Executive.

The Chief Executive is responsible for advising and informing the board on the implementation of agreed organisational strategy, service development and delivery, as well as any requisite changes to the policy, and practice for the group. He is supported by an Executive Team, which is responsible and accountable for the day-to-day management of clinical services, corporate and financial affairs, HR and business development.

The Chief Executive meets regularly with the Chair of Trustees and provides full details of any significant issues as they may arise between the formal meetings.

Risk management

The trustees have assessed the major risks to which the company and the group is exposed, in particular those related to the operations and finances of the company and the group, and are satisfied that systems and procedures are in place to mitigate our exposure to the major risks.

Funds held as custodian

The Charity works with other organisations in pursuit of its charitable objectives and holds funds as custodian on behalf of others.

During the year, MHC has administered funds on behalf of partner agencies and committees in order to 'host' and develop projects whose objectives are mental health care-related. These functions are part of MHC's role as an enabling organisation within the local mental health arena. Where we hold funds as custodian these are recognised as a liability in our balance sheet.

Our service users are sometimes unable to take full control of their own financial affairs and we are asked to receive their welfare benefits on their behalf. We hold residents' money separately from those of the charity and account to the residents and their carers for our actions. The funds are included within our cash deposits and the amounts owed to residents are held within creditors.

Voluntary Sector Advisory Group (VOLSAG)
This group consists of non-statutory service providers with an interest in mental health, alongside representatives from the statutory services. Over time, small donations (usually from Local Authority / Health Commissioners) have been made to VOLSAG in order to host events and aid the development of small mental health-related projects. MHC administers this fund on behalf of VOLSAG as a constituent member.

Members' liability

The members of the company and the group guarantee to contribute an amount not exceeding £1 to the assets of the charity in the event of winding up.

Employee involvement and employment of the disabled

MHC is committed to equal opportunities for all and gives full and fair consideration to job applications from disabled persons. Employees who are disabled are treated as equal to other employees, with regards to continuing employment, training, career development, and promotion.

MHC is committed to open and full engagement with employees. All new employees receive a comprehensive induction, regular supervision, and annual appraisal. All members of staff have access to the company website and intranet, and receive appropriate organisational and statutory training. Senior Managers and the Chief Executive make regular visits to all of the company's locations as part of their roles.

The response to our 2015/16 annual staff survey was overall very positive; 96% of MHC staff agreed that they would recommend the organisation as a place to work. The following areas are amongst those which received the highest satisfaction scores:

- job satisfaction
- opportunities to use initiative
- level of support from immediate colleagues
- level of responsibility given in their role

As a result of feedback from the previous survey, and recognising there is room for improvement, the following has been implemented during the year:

- further training was provided to managers on performance appraisal
- communication processes were improved with the introduction of:
  - communications briefing documents to be used by managers to brief staff, ensuring that key messages are delivered in a consistent and timely way
  - SharePoint, an enhanced Intranet site for access across the group providing information on group policies, feedback section for staff on issues that may impact upon them, communication and announcements page
- review of occupational health provider, which has resulted in a new contract being awarded offering more enhanced services accessible across the group
- an awards ceremony, recognising the achievements of staff, is to be held every two years once the organisational values are fully embedded across the group
- development of a management leadership programme

Appointment of auditors

A full review of our auditors was carried out this year, in accordance with best practice to ensure full independence and value for money, as no such review had been carried out previously. UNW LLP was appointed as auditor in March 2016 for the year ended 31 March 2016, following a full tender exercise including presentations and interviews. The quality of each submission was high with UNW scoring highly across all factors assessed. The trustees are sad to have ended the long relationship with Baker Tilly UK Audit LLP but looking forward to working with UNW LLP.

In accordance with the Companies Act 2006 a resolution proposing the reappointment of UNW LLP as auditor for the year ending 31 March 2017 will be put to the members.

Disclosure of information to auditors

Each of the persons who are trustees at the time when this trustees' report is approved has confirmed that:

- so far as that trustee is aware, there is no relevant audit information of which the charitable group's auditors are unaware, and
- that trustee has taken all the steps that ought to have been taken as a trustee in order to be aware of any relevant audit information and to establish that the charitable group's auditors are aware of that information.

This report was approved by the trustees on 31 August 2016 and signed on their behalf by:

Mrs E A Robinson, Chair
Trustee

Trustees' responsibilities statement
Year ended 31 March 2016

The trustees (who are also directors of Mental Health Concern for the purposes of company law) are responsible for preparing the trustees' report and the financial statements in accordance with applicable law and United Kingdom Accounting Standards (United Kingdom Generally Accepted Accounting Practice).

Company law requires the trustees to prepare financial statements for each financial year. Under company law the trustees must not approve the financial statements unless they are satisfied that they give a true and fair view of the state of affairs of the charitable company and the group and of the incoming resources and application of resources, including the income and expenditure, of the charitable group for that period. In preparing these financial statements, the trustees are required to:

- select suitable accounting policies and then apply them consistently;
- observe the methods and principles of the Charities SORP;
- make judgments and accounting estimates that are reasonable and prudent;
- prepare the financial statements on the going concern basis unless it is inappropriate to presume that the charitable group will continue in operation.

The trustees are responsible for keeping adequate accounting records that are sufficient to show and explain the charitable company and the group's transactions and disclose with reasonable accuracy at any time the financial position of the charitable company and the group and enable them to ensure that the financial statements comply with the Companies Act 2006. They are also responsible for safeguarding the assets of the charitable company and the group and hence for taking reasonable steps for the prevention and detection of fraud and other irregularities.

UNW LLP

Independent auditors' report to the members of Mental Health Concern

We have audited the financial statements of Mental Health Concern for the year ended 31 March 2016 set out on pages 23 to 51. The financial reporting framework that has been applied in their preparation is applicable law and United Kingdom Accounting Standards (United Kingdom Generally Accepted Accounting Practice).

This report is made solely to the charitable company's members, as a body, in accordance with Chapter 3 of Part 16 of the Companies Act 2006. Our audit work has been undertaken so that we might state to the charitable company's members those matters we are required to state to them in an auditors' report and for no other purpose. To the fullest extent permitted by law, we do not accept or assume responsibility to anyone other than the charitable company and its members, as a body, for our audit work, for this report, or for the opinion we have formed.

Respective responsibilities of trustees and auditors

As explained more fully in the trustees' responsibilities statement, the trustees (who are also the directors of the charitable company for the purposes of company law) are responsible for the preparation of the financial statements and for being satisfied that they give a true and fair view.

Our responsibility is to audit and express an opinion on the financial statements in accordance with applicable law and International Standards on Auditing (UK and Ireland). Those standards require us to comply with the Auditing Practices Board's Ethical Standards for Auditors.

Scope of the audit of the financial statements

An audit involves obtaining evidence about the amounts and disclosures in the financial statements sufficient to give reasonable assurance that the financial statements are free from material misstatement, whether caused by fraud or error. This includes an assessment of: whether the accounting policies are appropriate to the group's and the parent company's circumstances and have been consistently applied and adequately disclosed; the reasonableness of significant accounting estimates made by the trustees; and the overall presentation of the financial statements. In addition, we read all the financial and non-financial information in the trustees' report to identify material inconsistencies with the audited financial statements and to identify any information that is apparently materially incorrect based on, or materially inconsistent with, the knowledge acquired by us in the course of performing the audit. If we become aware of any apparent material misstatements or inconsistencies we consider the implications for our report.

UNW

Independent auditors' report to the members of Mental Health Concern

Opinion on financial statements

In our opinion the financial statements:

- give a true and fair view of the state of the group's and the parent charitable company's affairs as at 31 March 2016 and of the group's incoming resources and application of resources, including its income and expenditure, for the year then ended;
- have been properly prepared in accordance with United Kingdom Generally Accepted Accounting Practice; and
- have been prepared in accordance with the requirements of the Companies Act 2006.

Opinion on other matter prescribed by the Companies Act 2006

In our opinion the information given in the trustees' report, incorporating the group strategic report, for the financial year for which the financial statements are prepared is consistent with the financial statements.

Matters on which we are required to report by exception

We have nothing to report in respect of the following matters where the Companies Act 2006 requires us to report to you if, in our opinion:

- the parent charitable company has not kept adequate accounting records, or returns adequate for our audit have not been received from branches not visited by us; or
- the parent charitable company financial statements are not in agreement with the accounting records and returns; or
- certain disclosures of trustees' remuneration specified by law are not made; or
- we have not received all the information and explanations we require for our audit.

Andrew Wilson BA FCA (Senior Statutory Auditor)
for and on behalf of UNW LLP, Statutory Auditor

Chartered Accountants
Newcastle upon Tyne
31 August 2016

Mental Health Concern

(A company limited by guarantee)

Consolidated statement of financial activities (incorporating income and expenditure account) Year ended 31 March 2016

	Note	Unrestricted funds 2016 £	Restricted funds 2016 £	Total funds 2016 £	Total funds 2015 £
Income from:					
Donations	3	**3,120**	-	**3,120**	330
Other trading activities	4	**32,191**	-	**32,191**	27,816
Investments	5	**20,883**	-	**20,883**	21,696
Charitable activities:	6				
Mental Health activities		**8,507,825**	**201,168**	**8,708,993**	8,512,477
IAPT services		**10,022,775**	-	**10,022,775**	8,196,211
Other income		**9,703**	-	**9,703**	7,474
Total		**18,596,497**	**201,168**	**18,797,665**	16,766,004
Expenditure:					
Charitable activities	7	**18,565,263**	**173,721**	**18,738,984**	16,687,948
Total		**18,565,263**	**173,721**	**18,738,984**	16,687,948
Net income before investment gains		**31,234**	**27,447**	**58,681**	78,056
Share of profit in joint venture		**77,124**	-	**77,124**	49,210
Net income before transfers		**108,358**	**27,447**	**135,805**	127,266
Transfers between funds	22	**21,075**	**(21,075)**	-	-
Net income before other gains and losses		**129,433**	**6,372**	**135,805**	127,266
Losses on revaluations of fixed assets	13	**(376,515)**	-	**(376,515)**	-
Net movement in funds		**(247,082)**	**6,372**	**(240,710)**	127,266
Reconciliation of funds:					
Total funds at 1 April 2015		**10,728,179**	**93,905**	**10,822,084**	10,694,818
Total funds at 31 March 2016		**10,481,097**	**100,277**	**10,581,374**	10,822,084

All activities relate to continuing operations.

The Statement of Financial Activities includes all gains and losses recognised in the year.

The notes on pages 28 to 51 form part of these financial statements.

Mental Health Concern

(A company limited by guarantee)

Consolidated balance sheet
As at 31 March 2016

	Note	£	2016 £	£	2015 £
Fixed assets					
Intangible assets	12		**121,274**		165,619
Tangible assets	13		**9,005,366**		9,426,680
Investments in joint ventures					
-Share of gross assets		**143,081**		177,481	
-Share of gross liabilities		**(53,577)**		(83,283)	
Share of net assets	14		**89,504**		94,198
Investments in associates	14		**20,000**		20,000
			9,236,144		9,706,497
Current assets					
Stocks	16	**181,822**		222,883	
Debtors	17	**1,625,757**		1,191,435	
Cash at bank and in hand		**1,875,278**		2,105,029	
		3,682,857		3,519,347	
Creditors: amounts falling due within one year	18	**(2,088,396)**		(2,240,071)	
Net current assets			**1,594,461**		1,279,276
Total assets less current liabilities			**10,830,605**		10,985,773
Creditors: amounts falling due after more than one year	19		**(106,112)**		(141,108)
Deferred tax	20	**(21,603)**		(22,581)	
Provisions for Liabilities	21	**(121,516)**		-	
			(143,119)		(22,581)
Net assets			**10,581,374**		10,822,084
Charity Funds					
Restricted funds	22		**100,277**		93,905
Unrestricted funds (including Revaluation reserve of £6,187,630 (2015: £6,604,450))	22		**10,481,097**		10,728,179
Total funds			**10,581,374**		10,822,084

Consolidated balance sheet (continued)
As at 31 March 2016

The financial statements were approved by the trustees on 31 August 2016 and signed on their behalf by:

Mrs E A Robinson, Chair

The notes on pages 28 to 51 form part of these financial statements.

Registered company number 2073465

Mental Health Concern

(A company limited by guarantee)

Company balance sheet
As at 31 March 2016

	Note	£	2016 £	£	2015 £
Fixed assets					
Tangible assets	13		**8,943,886**		9,388,155
Investments	14		**1,025,339**		1,025,339
			9,969,225		10,413,494
Current assets					
Debtors	17	**570,611**		594,272	
Cash at bank		**1,626,045**		1,455,142	
		2,196,656		2,049,414	
Creditors: amounts falling due within one year	18	**(805,196)**		(825,046)	
Net current assets			**1,391,460**		1,224,368
Total assets less current liabilities			**11,360,685**		11,637,862
Creditors: amounts falling due after more than one year	19		**(106,112)**		(141,108)
Net assets			**11,254,573**		11,496,754
Charity Funds					
Restricted funds	22		**100,277**		93,905
Unrestricted funds (including Revaluation reserve of £6,187,630 (2015: £6,604,450))	22		**11,154,296**		11,402,849
Total funds			**11,254,573**		11,496,754

The financial statements were approved by the trustees on 31 August 2016 and signed on their behalf by:

Mrs E A Robinson, Chair

The notes on pages 28 to 51 form part of these financial statements.

Registered company number 2073465

Mental Health Concern

(A company limited by guarantee)

Consolidated cash flow statement
Year ended 31 March 2016

	Note	2016 £	2015 £
Cash flows from operating activities			
Net cash used in operating activities	24	**(163,988)**	(91,106)
Cash flows from investing activities:			
Dividends, interest and rents from investments		**20,883**	36,841
Proceeds from the sale of property, plant and equipment		**140,865**	-
Purchase of property, plant and equipment		**(195,795)**	(110,825)
Net cash used in investing activities		**(34,047)**	(73,984)
Cash flows from financing activities:			
Repayments of borrowings		**(31,716)**	(162,187)
Net cash used in financing activities		**(31,716)**	(162,187)
Change in cash and cash equivalents in the year		**(229,751)**	(327,277)
Cash and cash equivalents brought forward		**2,105,029**	2,432,306
Cash and cash equivalents carried forward	25	**1,875,278**	2,105,029

1. Accounting policies

1.1 Basis of preparation of financial statements

The financial statements have been prepared in accordance with Accounting and Reporting by Charities: Statement of Recommended Practice applicable to charities preparing their accounts in accordance with the Financial Reporting Standard applicable in the UK and Republic of Ireland (FRS 102) (effective 1 January 2015) - (Charities SORP (FRS 102)), the Financial Reporting Standard applicable in the UK and Republic of Ireland (FRS 102) and the Companies Act 2006. The financial statements are prepared in sterling which is the functional currency of the charity and rounded to the nearest £1.

Mental Health Concern meets the definition of a public benefit entity under FRS 102. Assets and liabilities are initially recognised at historical cost or transaction value unless otherwise stated in the relevant accounting policy.

The statement of financial activities (SOFA) and balance sheet consolidate the financial statements of the company and its subsidiary undertaking. The results of the subsidiary are consolidated on a line by line basis.

No separate SOFA has been presented for the company alone as permitted by section 408 of the Companies Act 2006.

1.2 Reconciliation with previous Generally Accepted Accounting Practice

In preparing these accounts, the trustees have considered whether in applying the accounting policies required by FRS 102 and the Charities SORP FRS 102 the restatement of comparative items was required.

No restatements to previously reported reserves were required. However, governance costs, previously shown as a separate expense category on the face of the Statement of Financial Activities, have now been reallocated to support costs within Charitable Activities, and as such the comparative figures have been restated. In addition, software development costs previously capitalised within fixed assets have been reclassified as software development costs within Intangible assets on the introduction of the new standards.

1.3 Company status

The company is a company limited by guarantee. The members of the company are the trustees named on page 1. In the event of the company being wound up, the liability in respect of the guarantee is limited to £1 per member of the company.

1. Accounting policies (continued)

1.4 Fund accounting

General funds are unrestricted funds which are available for use at the discretion of the trustees in furtherance of the general objectives of the company and which have not been designated for other purposes.

Restricted funds are funds which are to be used in accordance with specific restrictions imposed by donors or which have been raised by the company for particular purposes. The costs of raising and administering such funds are charged against the specific fund. The aim and use of each restricted fund is set out in the notes to the financial statements.

Investment income, gains and losses are allocated to the appropriate fund.

1.5 Income

All income is recognised once the company has entitlement to the income, it is probable that the income will be received and the amount of income receivable can be measured reliably.

Income on long-term contracts is calculated by reference to the value of the work performed to date as a proportion of total contract value. Foreseeable contract losses are provided for.

Income tax recoverable in relation to investment income is recognised at the time the investment income is receivable.

Other income is recognised in the period in which it is receivable and to the extent the goods have been provided or on completion of the service.

1.6 Expenditure

Expenditure is recognised once there is a legal or constructive obligation to make payment to a third party, it is probable that settlement will be required and the amount of the obligation can be measured reliably.

All expenditure is accounted for on an accruals basis. All expenses including support costs and governance costs are allocated to the applicable expenditure headings.

Support costs are those costs incurred directly in support of expenditure on the objects of the group.

1. Accounting policies (continued)

1.7 Basis of consolidation

The financial statements consolidate the accounts of Mental Health Concern and all of its subsidiary undertakings ('subsidiaries').

The company has taken advantage of the exemption contained within 408 of the Companies Act 2006 not to present its own income and expenditure account.

The income and expenditure account for the year dealt with in the accounts of the company was £256,013 (2015 - £46,795).

1.8 Intangible fixed assets and amortisation

Goodwill is the difference between amounts paid on the acquisition of a business and the fair value of the identifiable assets and liabilities. It is amortised to the statement of financial activities over its estimated economic life.

Software development costs are stated at cost less amortisation. Amortisation is calculated so as to write off the cost of an asset, less its estimated residual value, over the useful life of that asset. These costs are amortised over 3 years.

1.9 Tangible fixed assets and depreciation

All assets costing more than £5,000 are capitalised.

The policy with respect to impairment reviews of fixed assets is to review annually the continuing use of the assets held, and to carry out an impairment review if their continued use within the business is in doubt.

Tangible fixed assets are stated at cost or valuation less depreciation. Depreciation is provided at rates calculated to write off the cost or valuation of fixed assets, less their estimated residual value, over their expected useful lives on the following bases:

Freehold property	-	50 years to residual value
Leasehold property improvements	-	shorter of lease term or 10 years
Fixtures and fittings	-	1-10 years straight line
Office equipment	-	4 years straight line

Provision is made for any impairment.

1. Accounting policies (continued)

1.10 Revaluation of tangible fixed assets

The company has adopted the revaluation model to revalue items of property, plant and equipment whose fair value can be measured reliably. The revaluations shall be made with sufficient regularity to ensure that the carrying amount does not differ materially from that which would be determined using fair value at the end of the reporting period.

No depreciation is provided where assets are held at valuation.

The fair value of land and buildings is usually determined from market-based evidence by appraisal that is normally undertaken by professionally qualified valuers.

Revaluation gains and losses are recognised in other comprehensive income and accumulated in equity.

1.11Fixed asset investments

IIn the company balance sheet, investments in subsidiary undertakings are measured at cost less accumulated impairment losses.

Investments in joint ventures are stated at the company's share of net assets. The company's share of the profits or losses of the joint ventures is included in the statement of financial activities using the equity accounting basis.

1.12 Interest receivable

Interest on funds held on deposit is included when receivable and the amount can be measured reliably by the company; this is normally upon notification of the interest paid or payable by the Bank.

1.13 Operating leases

Rentals under operating leases are charged to the statement of financial activities on a straight line basis over the lease term.

1.14 Stocks and work in progress

Stocks are valued at the lower of cost and net realisable value after making due allowance for obsolete and slow-moving stocks. Cost includes all direct costs and an appropriate proportion of fixed and variable overheads.

Work in progress is valued on the basis of costs incurred on IAPT (Improving Access to Psychological Therapies) referrals which are ongoing at the year end. No element of incoming gains is included in the valuation of work in progress. Referrals relate to individuals' case files that have been opened and are in the process of being dealt with by staff of Concern Group.

1. Accounting policies (continued)

1.15 Debtors

Trade and other debtors are recognised at the settlement amount after any trade discount offered. Prepayments are valued at the amount repaid net of any trade discounts due.

1.16 Cash at bank and in hand

Cash at bank and in hand includes cash and short term highly liquid investments with a short maturity of three months or less from the date of acquisition or opening of the deposit or similar account.

1.17 Creditors and provisions

Creditors and provisions are recognised where the company has a present obligation resulting from a past event that will probably result in the transfer of funds to a third party and the amount due to settle the obligation can be measured or estimated reliably. Creditors and provisions are normally recognised at their settlement amount after allowing for any trade discounts due.

1.18 Financial instruments

Financial instruments are classified and accounted for as financial assets, financial liabilities or equity instruments, according to the substance of the contractual arrangement.

Financial instruments which are assets are stated at cost less any provision for impairment. Financial liabilities are stated at principal capital amounts outstanding at the period end. Issue costs relating to financial liabilities are deducted from the outstanding balance and are amortised over the period to the due date for repayment of the financial liability.

1. Accounting policies (continued)

1.19 Deferred taxation

Deferred tax represents the future tax consequences of transactions and events recognised in the financial statements of current and previous periods. It is recognised in respect of all timing differences, with certain exceptions. Timing differences are differences between taxable profits and total comprehensive income as stated in the financial statements that arise from the inclusion of income and expense in tax assessments in periods different from those in which they are recognised in the financial statements. Unrelieved tax losses and other deferred tax assets are recognised only to the extent that it is probable that they will be recovered against the reversal of deferred tax liabilities or other future taxable profits.

Deferred tax is measured using the tax rates and laws that have been enacted or substantively enacted by the balance sheet date that are expected to apply to the reversal of timing differences. Deferred tax on revalued non-depreciable tangible fixed assets is measured using the rates and allowances that apply to the sale of the asset.

1.20 Taxation

The company is considered to pass the tests set out in Paragraph 1 Schedule 6 of the Finance Act 2010 and therefore it meets the definition of a charitable company for UK corporation tax purposes. Accordingly, the company is potentially exempt from taxation in respect of income or capital gains received within categories covered by Chapter 3 Part 11 of the Corporation Tax Act 2010 or Section 256 of the Taxation of Chargeable Gains Act 1992, to the extent that such income or gains are applied exclusively to charitable purposes.

1.21 Pensions

The group operates a defined contribution pension scheme and the pension charge represents the amounts payable by the group to the fund in respect of the year.

The group also participates in a defined benefit pension scheme. The scheme is an unfunded scheme with benefits paid from current contributions, and as such no deficit on the scheme exists. Consequently the group has no liability to fund any historic deficit within the scheme. It is not possible to separately identify the assets of the scheme relating to the group, and as such it is accounted for as a defined contribution scheme.

2. Judgements in applying accounting policies and key sources of estimation uncertainty

The group enters into contracts with customers with varying contract periods. Management consider that the main judgement and source of estimation uncertainty arises from the estimation of progress within the specified services for amounts held as work in progress, and the assessment of the level of profitability around key contracts.

3. Voluntary income

	Total funds 2016 £	Total funds 2015 £
Donations	**3,120**	330

4. Income from other trading activities

	Total funds 2016 £	Total funds 2015 £
Office sublet	**29,591**	25,216
Room hire	**2,600**	2,600
	32,191	27,816

5. Investment income

	Total funds 2016 £	Total funds 2015 £
Bank interest receivable	**20,883**	21,696

6. Income from charitable activities

	Total funds 2016 £	Total funds 2015 £
Dementia Care, rehabilitation and recovery services	**6,744,316**	6,862,679
IAPT services provided under Insight Healthcare Limited	**10,022,775**	8,196,211
Supported housing services	**788,595**	716,246
Wellbeing services	**1,084,942**	758,311
Other mental health activities	**91,140**	175,241
	18,731,768	16,708,688

In 2016, of the total income from charitable activities, £201,168 (2015: £244,404) related to restricted funds and £18,530,600 (2015: £16,464,284) related to unrestricted funds.

7. Analysis of expenditure on charitable activities

	Unrestricted funds 2016 £	**Restricted funds 2016 £**	**Total funds 2016 £**	Total funds 2015 £
Dementia Care and recovery services	**8,051,323**	**173,721**	**8,225,044**	8,374,768
IAPT services	**9,808,343**	**-**	**9,808,343**	7,979,411
Supported housing services	**400,427**	**-**	**400,427**	240,055
Wellbeing services	**264,771**	**-**	**264,771**	67,629
Other activities	**40,399**	**-**	**40,399**	26,085
	18,565,263	**173,721**	**18,738,984**	16,687,948

In 2015, of the total expenditure, £ 16,473,614 was expenditure from unrestricted funds and £ 214,333 was expenditure from restricted funds.

8. Analysis of resources expended by activities

	Activities undertaken directly 2016 £	Support costs 2016 £	Total 2016 £	Total 2015 £
Dementia care and recovery services	**7,036,341**	**1,189,746**	**8,226,087**	8,374,768
IAPT services	**9,102,135**	**706,208**	**9,808,343**	7,979,411
Supported housing services	**266,026**	**134,401**	**400,427**	240,055
Wellbeing services	**85,940**	**178,831**	**264,771**	67,629
Other activities	**26,093**	**14,306**	**40,399**	26,085
Total	**16,516,535**	**2,223,492**	**18,740,027**	16,687,948

9. Support costs

	Mental Health services £	IAPT services £	Supported housing se £	Wellbeing services £
Office costs	**404,323**	**55,619**	**-**	**-**
Other costs	**70,216**	**85,377**	**-**	**-**
Governance costs	**1,200**	**6,000**	**-**	**-**
Wages and salaries	**712,964**	**559,212**	**134,401**	**178,831**
	1,188,703	**706,208**	**134,401**	**178,831**

	Other mental health £	Total 2016 £	Total 2015 £
Office costs	**-**	**459,942**	349,367
Other costs	**-**	**155,593**	152,555
Governance costs	**-**	**7,200**	16,830
Wages and salaries	**14,306**	**1,599,714**	1,556,648
	14,306	**2,222,449**	2,075,400

10. Net Income

Net income is stated after charging/(crediting):

	2016	2015
	£	£
Depreciation of tangible fixed assets:		
-owned by the company	**42,798**	35,600
Staff pension contributions	**533,413**	523,191
Operating lease rentals:		
- other operating leases	**289,305**	244,294
Bank loan interest	**8,241**	12,710
Loss on disposal of fixed assets	**34,655**	-
Auditor's remuneration		
Audit of these financial statements	**12,500**	13,750
Amounts receivable by the auditors and their associates in respect of:		
Other services relating to taxation	**1,000**	950

11. Staff costs

Staff costs were as follows:

	2016	2015
	£	£
Wages and salaries	**11,808,178**	11,215,216
Social security costs	**959,953**	428,179
Other pension costs	**533,413**	134,893
	13,301,544	11,778,288

The average monthly number of employees was 578 (2015: 462) and the average monthly number of employees during the year expressed as full time equivalents was as follows (including casual and part-time staff):

	2016	2015
	No.	No.
Nursing, therapy, support and administration staff	**577**	462

The number of higher paid employees was:

	2016	2015
	No.	No.
In the band £60,001 - £70,000	**1**	1
In the band £70,001 - £80,000	**1**	0
In the band £90,001 - £100,000	**1**	1
	3	2

Key management personnel are deemed to be those having authority and responsibility, delegated to them by the trustees for planning, directing and controlling the activities of the charity, as listed on page 1. During the year their total remuneration, including employee benefits, amounted to £450,195 (2015: £419,353).

12. Intangible fixed assets

Group	Software developm't costs £	Goodwill £	Total £
Cost			
At 1 April 2015	**165,619**	**506,594**	**672,213**
Additions	**16,291**	**-**	**16,291**
At 31 March 2016	**181,910**	**506,594**	**688,504**
Amortisation			
At 1 April 2015	**-**	**506,594**	**506,594**
Charge for the year	**60,636**	**-**	**60,636**
At 31 March 2016	**60,636**	**506,594**	**567,230**
Net book value			
At 31 March 2016	**121,274**	**-**	**121,274**
At 31 March 2015	165,619	-	165,619

13. Tangible fixed assets

Group	Freehold property £	Long-term leasehold property £	Fixtures and fittings £	Office equipment £	Total £
Cost or valuation					
At 1 April 2015	**8,907,500**	**593,088**	**79,833**	**257,038**	**9,837,459**
Additions	**-**	**98,422**	**43,295**	**37,789**	**179,506**
Disposals	**(180,000)**	**(1,419)**	**-**	**-**	**(181,419)**
Revaluation surplus/(deficit)	**(405,000)**	**28,397**	**-**	**-**	**(376,603)**
At 31 March 2016	**8,322,500**	**718,488**	**123,128**	**294,827**	**9,458,943**
Depreciation					
At 1 April 2015	**-**	**154,883**	**79,833**	**176,063**	**410,779**
Charge for the year	**-**	**27,964**	**-**	**14,834**	**42,798**
At 31 March 2016	**-**	**182,847**	**79,833**	**190,897**	**453,577**
Net book value					
At 31 March 2016	**8,322,500**	**535,641**	**43,295**	**103,930**	**9,005,366**
At 31 March 2015	8,907,500	438,205	-	80,975	9,426,680

The charity's freehold and leasehold property was revalued in January 2016 on an existing use basis by independent valuers, Sanderson Weatherall, Chartered Surveyors, and incorporated into the financial statements as at that date.

If the land and buildings had not been included at valuation they would have been included under the historical cost convention as follows:

Group	2016 £	2015 £
Cost	**3,428,149**	3,475,072
Accumulated depreciation	**(885,402)**	(835,419)
Net book value	**2,542,747**	2,639,653

Company	Freehold property £	Long-term leasehold property £	Fixtures and fittings £	Office equipment £	Total £
Cost or valuation					
At 1 April 2015	**8,907,500**	**588,518**	**15,578**	**42,450**	**9,554,046**
Additions	**-**	**98,422**	**43,295**	**-**	**141,717**
Disposals	**(180,000)**	**(1,419)**	**-**	**-**	**(181,419)**
Revaluation surplus/(deficit)	**(405,000)**	**28,397**	**-**	**-**	**(376,603)**
At 31 March 2016	**8,322,500**	**713,918**	**58,873**	**42,450**	**9,137,741**
Depreciation					
At 1 April 2015	**-**	**150,313**	**15,578**	**-**	**165,891**
Charge for the year	**-**	**27,964**	**-**	**-**	**27,964**
At 31 March 2016	**-**	**178,277**	**15,578**	**-**	**193,855**
Net book value					
At 31 March 2016	**8,322,500**	**535,641**	**43,295**	**42,450**	**8,943,886**
At 31 March 2015	8,907,500	438,205	-	42,450	9,388,155

The charity's freehold and leasehold property was revalued in January 2016 on an existing use basis by independent valuers, Sanderson Weatherall, Chartered Surveyors, and incorporated into the financial statements as at that date.

If the land and buildings had not been included at valuation they would have been included under the historical cost convention as follows:

Company	2016 £	2015 £
Cost	**3,428,149**	3,475,072
Accumulated depreciation	**(885,402)**	(835,419)
Net book value	**2,542,747**	2,639,653

14. Fixed asset investments

Group	Share of joint venture £	Investments in participating interests £	Total £
Fair value			
At 1 April 2015	94,198	20,000	114,198
Movement in share of net assets	(4,694)	-	(4,694)
At 31 March 2016	89,504	20,000	109,504

Company	Shares in group undertakings £	Investments in participating interests £	Total £
Fair value			
At 1 April 2015 and 31 March 2016	1,005,339	20,000	1,025,339
Historical cost	1,025,339	20,000	1,045,339

15. Principal subsidiaries, joint ventures and associates

a. Principal subsidiaries

Company name	Country	Percentage Shareholding	Description
Insight Healthcare Limited	United Kingdom	100	Provision of counselling and psychological therapy services
MHC Resources Limited	United Kingdom	100	Dormant

b. Principal joint ventures and associates

Company name	Country	Percentage Shareholding	Description
MHC/NTW LLP	United Kingdom	50	Joint venture with Northumberland, Tyne and Wear NHS Foundation Trust
Northern Inclusion Consortium	United Kingdom	25	Partnership arrangement with other north east organisations.

Analysis of share of joint venture and associate results

	MHC/NTW LLP		Northern Inclusion Consortium Limited	
	2016	2015	**2016**	2015
	£	£	**£**	£
Balance sheet				
Share of current assets	**143,081**	177,481	**23,320**	19,181
Share of gross liabilities	**(53,578)**	(83,283)	**(10,101)**	(1,083)
Share of net assets	**89,503**	**94,198**	13,219	18,098
Profit and loss account				
Share of income	**654,524**	545,656	**2,954**	12,722
Share of expenditure	**(577,401)**	(496,446)	**(12,356)**	(14,625)
Share of profit	**77,123**	**49,210**	(9,402)	(1,903)

16. Stocks

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Work in progress	**181,822**	222,883	**-**	-

17. Debtors

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Trade debtors	**894,631**	579,741	**121,139**	64,288
Amounts owed by group undertakings	**121,679**	-	**115,895**	307,289
Amounts owed by undertakings in which the company has a participating interest	**19,783**	1,491	**58,882**	1,491
Other debtors	**141,384**	67,608	**123,750**	54,149
Prepayments and accrued income	**448,280**	542,595	**150,945**	167,055
	1,625,757	1,191,435	**570,611**	594,272

18. Creditors:
Amounts falling due within one year

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Bank loans and overdrafts	**33,666**	31,716	**33,666**	31,716
Trade creditors	**557,291**	579,366	**233,357**	265,049
Amounts owed to group undertakings	**121,679**	-	**17,416**	1,852
Corporation tax	**61**	-	**-**	-
Other taxation and social security	**666,844**	480,301	**121,142**	134,046
Other creditors	**175,536**	167,848	**132,189**	114,757
Accruals and deferred income	**533,319**	980,840	**267,426**	277,626
	2,088,396	2,240,071	**805,196**	825,046

	Group	Company
	£	**£**
Deferred income		
Deferred income at 1 April 2015	**923,665**	**168,796**
Resources deferred during the year	**385,053**	**180,051**
Amounts released from previous years	**(923,665)**	**(168,796)**
Deferred income at 31 March 2016	**385,053**	**180,051**

Deferred income comprises PCT core funding and IAPT service receipts relating to future accounting periods.

19. Creditors:
Amounts falling due after more than one year

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Bank loans	**86,734**	120,400	**86,734**	120,400
Accruals and deferred income	**19,378**	20,708	**19,378**	20,708
	106,112	141,108	**106,112**	141,108

Included within the above are amounts falling due as follows:

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Between one and two years				
Bank loans	**33,667**	33,667	**33,667**	33,667
Between two and five years				
Bank loans	**53,067**	86,733	**53,067**	86,733

The bank borrowings are secured by fixed and floating charges over the current and future assets of Mental Health Concern. Loans due within five years are repayable in monthly instalments and bear interest at a rate of 5.985%. The propery to which the loan relates is Oakwell.

20. Deferred taxation

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
At beginning of year	**22,581**	2,463	**-**	-
(Released during)/charge for the year	**(978)**	20,118	**-**	-
At end of year	**21,603**	22,581	**-**	-

The provision for deferred taxation is made up as follows:

	Group		Company	
	2016	2015	**2016**	2015
	£	£	**£**	£
Accelerated capital allowances	**21,603**	22,581	**-**	-

21. Provisions

Group	**Provisions £**
At 1 April 2015	-
Additions	**121,516**
At 31 March 2016	**121,516**

Provisions

The provision relates to foreseeable losses on a contract within Insight Healthcare.

The Company has no provisions

22. Statement of funds

	Brought Forward £	Incoming resources £	Resources Expended £	Transfers in/out £	Gains/ (Losses) £	Carried Forward £
Unrestricted funds						
General unrestricted funds	4,123,729	18,596,497	(18,565,263)	21,075	117,429	4,293,467
Revaluation reserve	6,604,450	-	-	-	(416,820)	6,187,630
	10,728,179	18,596,497	(18,565,263)	21,075	(299,391)	10,481,097
Restricted funds						
Fulfilling lives	14,165	117,229	(94,751)	(12,640)	-	24,003
Launchpad	42,468	58,816	(62,762)	(5,824)	-	32,698
North Tyneside User Development reserve	32,436	25,123	(16,208)	(2,611)	-	38,740
Nicholas Milward donated funds	4,836	-	-	-	-	4,836
	93,905	201,168	(173,721)	(21,075)	-	100,277
Total of funds	10,822,084	18,797,665	(18,738,984)	-	(299,391)	10,581,374

Launchpad Social Fund
Lobbying project to ensure interests of mentally ill persons are represented to Health and Social Care Authorities. Launchpad is an ongoing project with funding from the CCG.

North Tyneside User Development
These funds are received to develop and support mental health user involvement in mental health services. This is an ongoing project with funding to support staff costs and activities.

Nicholas Milward Fund
The Nicholas Milward Funds are funds to be used in Mental Health Care Services specifically within the service at Alderwood.

Big Lottery Fund – Funded posts
The Big Lottery Fund have supported the Fulfilling Lives service established in the year in collaboration with other Third sector providers. The funds support the ongoing cost of service navigators.

23. Analysis of net assets between funds

	Unrestricted funds 2016 £	Restricted funds 2016 £	Total funds 2016 £	Total funds 2015 £
Tangible fixed assets	**9,005,366**	-	**9,005,366**	9,426,678
Intangible fixed assets	**121,274**	-	**121,274**	165,619
Fixed asset investments	**109,504**	-	**109,504**	114,198
Current assets	**3,582,578**	**100,277**	**3,682,855**	3,519,347
Creditors due within one year	**(2,088,394)**	-	**(2,088,394)**	(2,240,069)
Creditors due in more than one year	**(106,112)**	-	**(106,112)**	(141,108)
Provisions for liabilities and charges	**(143,119)**	-	**(143,119)**	(22,581)
	10,481,097	**100,277**	**10,581,374**	10,822,084

24. Reconciliation of net movement in funds to net cash flow from operating activities

	2016 £	2015 £
Net income for the year (as per Statement of financial activities)	**135,805**	127,266
Adjustment for:		
Depreciation charges	**103,433**	35,600
Share of profit in joint venture	**(77,125)**	(49,210)
Dividends, interest and rents from investments	**(20,883)**	(8,987)
Loss on the sale of fixed assets	**40,305**	-
Decrease in work in progress	**41,061**	23,365
(Increase)/decrease in debtors	**(83,615)**	61,809
Decrease in creditors	**(302,969)**	(280,949)
Net cash used in operating activities	**(163,988)**	(91,106)

25. Analysis of cash and cash equivalents

	2016 £	2015 £
Cash in hand	**1,875,278**	2,105,029
Total	**1,875,278**	2,105,029

26. Pension commitments

The group operates a defined contributions pension scheme. The assets of the scheme are held separately from those of the group in an independently administered fund. The pension cost charge represents contributions payable by the group to the fund and amounted to £163,723 (2015 - £161,584). Contributions totalling £16,037 (2015 - £19,411) were payable to the fund at the balance sheet date and are included in creditors.

The group also participates in the NHS defined benefit scheme. It is not possible to separately identify the assets of the scheme relating to the group, and as such it is accounted for as a defined contribution scheme. The pension cost charge represents contributions payable by the group to the fund and amounted to £369,690 (2015 - £361,607). Contributions totalling £30,688 (2015 - £26,850) were payable to the fund at the balance sheet date and are included in creditors.

27. Operating lease commitments

At 31 March 2016 the group had future minimum lease payments under non-cancellable operating leases as follows:

	Land and buildings		Other	
	2016	2015	**2016**	2015
Group	**£**	£	**£**	£
Within 1 year	**478,871**	344,044	**51,183**	63,599
Between 2 and 5 years	**1,805,914**	918,864	**103,789**	48,256
After more than 5 years	**355,491**	167,946	**-**	-
Total	**2,640,276**	1,430,854	**154,972**	111,855

At 31 March 2016 the group had future minimum lease payments under non-cancellable operating leases as follows:

	Land and buildings		Other	
	2016	2015	**2016**	2015
Company	**£**	£	**£**	£
Within 1 year	**228,945**	245,445	**38,263**	51,897
Between 2 and 5 years	**886,180**	550,900	**95,062**	27,847
After more than 5 years	**125,557**	77,557	**-**	-
Total	**1,240,682**	873,902	**133,325**	79,744

28. Trustees' benefits: advances, credit and guarantees

The charity paid director's and officers' liability insurance amounting to £822 (2015: £822).

29. Related party transactions

The charity has taken exemption contained within FRS 102 for disclosing any transactions with entities which are part of the Mental Health Concern Group as consolidated financial statements are prepared.

During the year the charity received £nil from Northern Inclusion Consortium (2015: £480) for services of which £nil (2015: £13) was included within debtors at the year end.

30. Controlling party

The ultimate control of the charity rests with the board of trustees.

31. Custodian Funds

At the end of the year the charity was holding £36,000 of funds on behalf of partner agencies and committees.